CHAPTER 06

Sexual Violence, Harassment, and Exploitation

A survivor-centred educational chapter for Rwanda | English edition | Version 1.1 | 11 November 2026

Editorial status: Evidence-informed educational draft; not a clinical, forensic, safeguarding, or legal protocol. GBV specialist, clinical, child-protection, legal and language reviews are required before publication. All scenarios are fictional.

How to use this chapter

This chapter is written for adults, educators, community members, and service navigators. It explains forms of sexual violence and exploitation, what makes consent meaningful, how to respond safely, and where to seek support. Readers may skip examples or activities if they feel uncomfortable. No one needs to share personal experiences to complete the chapter.

Learning outcomes: After reading, a learner should be able to recognize coercion and abuse of authority; explain sexual harassment and exploitation; identify safer first responses; understand time-sensitive healthcare; and describe Rwanda-based support options.

1. Understanding sexual violence

Sexual violence refers to sexual acts, attempted sexual acts, or other sexual conduct carried out without valid consent, including through force, threats, intimidation, coercion, exploitation of vulnerability, or abuse of power. It can happen between strangers, acquaintances, relatives, colleagues, partners, or spouses. It may occur in person or through technology. Sexual violence is never justified by a person’s clothing, relationship history, social status, or previous consent. [1, 2]

Sexual violence is not limited to physically violent assault. It may involve unwanted sexual touching, forced or coerced sexual activity, attempted sexual assault, sexual exploitation, or non-consensual creation or sharing of sexual images. The exact legal definitions and offences differ by jurisdiction and must be checked against Rwandan law when legal information is published.

2. Consent: what makes agreement meaningful?

Consent is a freely given, informed, specific and ongoing agreement to a particular activity. Someone may change their mind. Being married, dating, accepting a gift, going to someone's room, or agreeing to one activity does not imply agreement to another. Silence, freezing, or absence of physical resistance is not by itself consent. [1, 2]

When agreement is not freely given

- A threat is made: for example, a supervisor suggests a student will fail an assessment unless they accept a sexual demand.
- Someone is unable to make an informed decision, for example due to unconsciousness or incapacitation.
- A person agrees because they fear injury, humiliation, losing a job, housing, essential support, or another serious consequence.
- A power imbalance is exploited: an authority figure controls grades, employment, aid, healthcare, or access to services.

The age and capacity of a child require separate safeguarding and legal rules. A child's apparent agreement does not remove the need for child protection and legal assessment.

3. Sexual harassment

Sexual harassment includes unwelcome sexual comments, requests, messages, gestures, images, touching, or other sexual conduct that violates dignity or creates an intimidating, hostile, humiliating, or offensive environment. It can occur in schools, workplaces, public transport, markets, health facilities, community organizations, or online spaces.

Two recurring patterns

Pressure linked to rewards or penalties: A person with authority requests sexual contact in exchange for a grade, placement, promotion, contract, or service. Hostile environment: Repeated sexual jokes, unwanted advances or degrading conduct make studying or working unsafe. A single serious incident can also warrant attention.

Examples: A lecturer repeatedly sends sexual messages to a student after being asked to stop; a manager threatens to reduce shifts following a rejected advance; commuters make unwanted sexual contact on a bus. The person experiencing harassment is not responsible for managing the offender's behavior.

4. Sexual exploitation and abuse of authority

Sexual exploitation involves abusing a position of vulnerability, differential power, or trust for sexual purposes, including situations in which a person seeks sexual access or advantage by controlling something the other person needs. A person may appear to agree while facing severe inequality in available choices. [3]

An aid worker who conditions assistance on sexual acts, a recruiter who demands sexual favors for employment, or a caretaker who uses dependence to pressure someone are illustrative forms of exploitation. Such conduct may also breach safeguarding obligations, professional ethics, and criminal law.

Why power matters

Power can come from grades, money, employment, legal or immigration status, access to food and shelter, professional authority, social influence, physical strength, or control over confidential information. Ethical organizations must define prohibitions, reporting routes, retaliation protections, and independent responses instead of leaving an affected person to negotiate with the person holding power.

5. Sexual coercion and intimidation

Coercion is pressure that undermines a person’s freedom to choose. It can include threats of violence, blackmail, threats to disclose intimate images, persistent pressure after refusal, threats to withdraw financial support, manipulation of dependency, or abuse of institutional authority. Coercion can occur without visible injury.

A healthy boundary is respected without argument, retaliation, or punishment. Repeated requests after a refusal, monitoring a person’s movements, or threatening their reputation should be treated as concerning behavior rather than romantic persistence.

6. Technology-facilitated sexual harm

Sexual harassment and coercion may occur through messaging, social networks, devices, and digital services. Examples include unsolicited sexual messages, demands for intimate photos, threats to publish images, sharing private sexual images without consent, impersonation, location-based stalking, or harassment across multiple accounts.

A person can consent to taking a photograph without consenting to its publication. Consent to share one message or image does not authorize redistribution. People experiencing digital abuse may need a tailored safety plan, especially if an offender can access their device or accounts.

Safer first considerations

- If safe, use a device or account the offender cannot access when seeking support.
- Avoid confronting the person or changing settings abruptly if those actions could increase danger.
- If safe and desired, preserve relevant messages or links without redistributing intimate content. For content involving children, seek qualified reporting guidance and do not save or share illegal imagery.
- Ask a trusted specialist or relevant authority about privacy, reporting, and evidence options.

7. Effects on wellbeing and daily life

Responses to sexual violence differ. Some survivors feel fear, anger, shame, sadness, numbness, or confusion; others may initially show little visible distress. Possible effects include injury, sexually transmitted infections, unintended pregnancy, sleep problems, traumatic stress, difficulty concentrating, withdrawal, and disruptions to education, work, and relationships. These are possible outcomes, not inevitable outcomes or criteria for believing someone.

People may delay disclosing because of fear, dependence, stigma, threats, family pressure, lack of safe services, or uncertainty about confidentiality. A delayed disclosure or fragmented memory does not prove an account false. Support should not depend on whether someone reports to police.

8. Responding to a disclosure: first-line support

The WHO clinical handbook emphasizes attention to immediate emotional needs, physical needs, ongoing safety, and longer-term support. The LIVES approach summarizes listening, inquiring about needs and concerns, validating, enhancing safety, and supporting access to services. These are principles for trained responders and should not be confused with conducting an investigation. [1]

Supportive words and actions

- Listen without interruption, interrogation, disbelief, or blame. A person is entitled to pause or stop.
- Acknowledge the experience: “Thank you for telling me. You deserve to be safe.”
- Ask about immediate needs: “Are you safe right now? Would you like help finding medical care or another support service?”
- Explain any limits to confidentiality before collecting sensitive details, especially when a child is involved.
- Offer choices. Do not pressure someone to reconcile, confront the offender, make a public disclosure, or take a specific legal step.

Avoid these responses

Do not ask why the survivor was in a location, why they did not fight, or why they waited to disclose. Do not request a repeated account merely to satisfy curiosity, offer guarantees about outcomes, or promise absolute secrecy. Do not mediate between survivor and alleged offender as a substitute for protection and professional response.

9. Urgent healthcare after sexual assault

Time matters: If sexual assault was recent, seek medical care as soon as safely possible, ideally immediately. HIV post-exposure prophylaxis (PEP), when clinically indicated, should start as soon as possible and no later than 72 hours after a potential exposure. Emergency contraception may also be time-sensitive; a clinician should advise on available methods and their effective windows. Even after these windows, medical care and support remain important. [4, 5]

Healthcare assessment may include treatment of injuries, assessment for HIV PEP, prevention or testing for sexually transmitted infections, emergency contraception where appropriate, other reproductive healthcare, psychological first-line support, and discussion of forensic documentation. The survivor should receive information and be asked for consent to examinations and procedures, subject to applicable safeguarding and legal requirements. [1, 4]

Do not delay medical care because you are unsure whether to report to police, or because clothing was changed or washing has occurred. Evidence options can be discussed by trained local providers. If someone is bleeding heavily, has severe injuries, is unconscious, or faces imminent danger, emergency care is the priority.

10. Safeguarding children and other vulnerable people

Children require an age-appropriate, trained response that prioritizes protection, avoids suggestive questioning, and follows applicable mandatory reporting, consent, and guardian rules. An accompanying adult is not always a safe guardian. Avoid asking the child to repeat a detailed account; record only essential facts according to authorized procedures and refer promptly to designated protection professionals.

People with disabilities, displaced persons, people dependent on caregivers, and others facing barriers may need accessible communication, interpreters, safe transport, or independent advocacy. Never assume that a survivor cannot make decisions merely because they need support.

11. Rwanda: care and referral pathways

Rwanda’s Isange One Stop Centres provide integrated services for GBV survivors, bringing together healthcare, psychosocial support, investigation and legal pathways. Government sources describe services as free and available through the national network. [6, 7]

Published contacts (verify before public deployment)

- Isange One Stop Centre: 3029 (listed by RIB and UNHCR Rwanda).
- Rwanda Investigation Bureau: 3512 for domestic/GBV reporting; 116 for child abuse; 166 for emergencies or crime information (RIB contact page).
- Other GBV referral information: MIGEPROF lists 9059 and Gender Monitoring Office 5798.

Important: UNHCR Rwanda notes that approaching an Isange Centre may lead to a report to law enforcement. Service users should ask how information will be shared and what confidentiality or child-protection obligations apply. Do not promise that a particular referral is anonymous or will avoid police involvement. [6, 7, 8]

12. Prevention in schools, workplaces, and institutions

Preventing sexual harassment and exploitation requires clear rules and accountable systems, not only individual awareness. Educational institutions and employers should prohibit sexual demands linked to grades, placement, aid, recruitment, or promotion; train staff; provide confidential and accessible reporting channels; protect complainants and witnesses from retaliation; and arrange independent, timely, fair responses.

A safe learning environment includes respectful boundaries, transparent assessment procedures, safeguarding responsibilities, and accessible support for students. A workplace should establish clear behavioral expectations and make reporting possible without forcing disclosure to the alleged offender's supervisor.

13. Fictional learning scenarios

Scenario A: University placement

A supervisor privately suggests an internship evaluation will improve if a student agrees to a sexual relationship. The student feels unable to refuse because graduation depends on the placement. Analysis: This is an abuse-of-authority concern. A trustworthy institutional reporting channel, protection from retaliation, and independent support are important; the student should not be expected to negotiate with the supervisor.

Scenario B: Workplace messages

A colleague sends repeated sexual messages after being asked to stop. A team leader dismisses the situation as a joke. Analysis: Unwelcome sexual communication can constitute harassment. Institutional responsibility includes safe reporting and proportionate responses.

Scenario C: Digital blackmail

A former partner threatens to send an intimate image to a family group unless the person agrees to meet. Analysis: Threats to distribute images are coercive and can heighten risk. The person may need digital safety support and confidential professional advice rather than being told to confront the former partner.

Scenario D: Recent assault

An adult tells a trusted friend that they were sexually assaulted the previous evening but do not want to report the incident yet. Analysis: The friend can explain that medical assessment is time-sensitive, offer to help reach an Isange Centre or hospital, and avoid pressuring the person into disclosure. Reporting-related confidentiality limits should be explained accurately by providers.

Scenario E: Child safeguarding

A pupil says an adult at school has repeatedly requested sexual contact and threatened punishment if they tell anyone. Analysis: The priority is immediate protection and referral to trained child-protection responders. Adults should avoid investigating independently or promising secrecy.

14. Guided educational activities

Activity 1: Recognize the power imbalance

In pairs or individually, identify who controls a resource in Scenario A, what consequence is being threatened, and why an apparent "yes" may not represent freely given consent. Suggested response: The supervisor controls evaluation; the student risks an educational penalty; this undermines free choice.

Activity 2: Practice supportive language

Rewrite “Why did you go there?” as a survivor-centred question. Suggested response: “What would help you feel safer right now?” Do not ask the learner to reveal personal experiences.

Activity 3: Choose a safer digital response

For Scenario C, describe two considerations before changing device settings or messaging the threatening person. Suggested response: Evaluate whether the person monitors the device, and identify a safer device or professional support contact.

Activity 4: Sort urgent and non-urgent needs

Classify the following: severe injury; a need for HIV exposure assessment after yesterday’s assault; a request for information about school policy. Suggested response: Injury and recent exposure warrant urgent healthcare; the policy question needs a safe institutional information channel.

Activity 5: Map a referral without assumptions

Using Rwanda’s publicly listed contacts, outline a referral that offers choice and explains possible information-sharing. Suggested response: Ask whether the person is safe and wants medical/support options, share the Isange contact 3029, explain that providers can clarify reporting obligations, and support safe access without making confidentiality promises.

15. Knowledge assessment with answer explanations

1. Does a previous agreement imply consent to future sexual activity?

Answer: No. Consent is specific and ongoing.

2. Can a threat to fail a student undermine consent?

Answer: Yes. It exploits unequal power and threatens a serious consequence.

3. Is sexual harassment limited to physical contact?

Answer: No. Unwelcome sexual messages, comments and other conduct can constitute harassment.

4. Can sharing an intimate image without permission cause sexual harm?

Answer: Yes. Image-based abuse violates privacy and can involve coercion.

5. Should a friend demand a detailed account from a survivor?

Answer: No. Listen, validate, and offer options without interrogation.

6. What is the critical HIV PEP window?

Answer: As soon as possible, ideally within 24 hours and no later than 72 hours after potential exposure, based on clinical assessment.

7. Does seeking help always guarantee no police reporting?

Answer: No. Applicable law and service processes may create reporting or information-sharing obligations.

8. Is medical care worthwhile after 72 hours?

Answer: Yes. Injury care, STI assessment, reproductive care and emotional support may still be needed.

9. Should teachers privately investigate suspected child sexual abuse?

Answer: No. Follow approved safeguarding pathways and avoid repeated questioning.

10. What should institutions provide?

Answer: Clear prevention policy, accessible reporting, retaliation protections and independent action.

16. Key terms

Consent: Freely given, informed and specific agreement that can be withdrawn.

Sexual coercion: Pressure, threats or exploitation that undermines a free choice.

Sexual harassment: Unwelcome sexual conduct that violates dignity or creates an intimidating or offensive environment.

Sexual exploitation: Abuse of vulnerability, power or trust for sexual purposes.

Safeguarding: Policies and actions intended to prevent and respond to harm, especially involving children and vulnerable people.

PEP: Time-sensitive antiretroviral medication used after a possible HIV exposure when clinically indicated.

Survivor-centred support: Support grounded in dignity, safety, privacy, informed choice and appropriate services.

Forensic documentation: Professional medical and evidentiary records collected under applicable procedures and consent requirements.

17. References and further reading

[1] World Health Organization. Health care for women subjected to intimate partner violence or sexual violence: a clinical handbook (2014). https://www.who.int/publications-detail-redirect/WHO-RHR-14.26

[2] World Health Organization. Responding to intimate partner violence and sexual violence against women: WHO clinical and policy guidelines (2013). https://www.who.int/publications/i/item/9789241548595

[3] UNHCR. Sexual exploitation and abuse prevention: definitions and safeguarding resources. https://www.unhcr.org/what-we-do/how-we-work/tackling-sexual-exploitation-abuse-and-harassment

[4] World Health Organization. Guidelines for HIV post-exposure prophylaxis (2024). https://www.who.int/publications/i/item/9789240095137

[5] Rwanda Ministry of Health. Guidelines for HIV Prevention, Treatment and Care (2022), sexual assault section. https://files-hivpreventioncoalition.unaids.org/gpc/attachments/final-guidelines-for-hiv-prevention-treatment-and-care-in-rw

anda-2022-c.pdf

[6] Rwanda Investigation Bureau. Official contact numbers. https://www.rib.gov.rw/contact-us

[7] Ministry of Gender and Family Promotion. Family promotion and child rights protection / Isange One Stop Centres. https://www.migeprof.gov.rw/family-promotion-and-child-right-protection

[8] UNHCR Rwanda. Gender-based violence services and Isange referral information. https://help.unhcr.org/rwanda/services/sgbv/

[9] Rwanda Gender Monitoring Office. Annual Report 2024-2025 (Isange service monitoring). https://www.gmo.gov.rw/fileadmin/user_upload/GMO/Publications/Annual_Reports/ANNUAL_REPORT_2024-2025.pdf

18. Publication and specialist review checklist

- Clinical lead confirms urgent healthcare and medication timing language.
- GBV/safeguarding specialist reviews all cases, consent language and first-response principles.
- Rwandan legal reviewer checks reporting obligations, child protection, and legal terminology.
- Referral owner tests every published telephone number and confirms service availability.
- Kinyarwanda and French reviewers verify meaning with local survivors’ safety considerations.
- Accessibility review checks plain-language readability and screen-reader compatibility.
- Content owner records approval, version, next review date, and any corrections.

End of Chapter 06. This educational material does not replace clinical care, survivor advocacy, emergency services or legal advice.